Trastuzumab deruxtecan (Enhertu) · every 3 weeks

One infusion on Day 1 of a 21-day cycle. The next infusion is normally three weeks later. A delayed appointment is rescheduled by the team; patients do not calculate catch-up doses.

HER2-directed antibody–drug conjugate · selected medicine-specific effects. Full details and sources are in the complete reference.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 New cough or breathlessness

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Possible lung inflammation needs urgent assessment at any time; do not wait for a scan appointment.

2 Heart pumping function

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Keep baseline and 12-weekly echo/MUGA appointments. Report breathlessness, chest symptoms or new swelling.

3 Nausea and intake

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Nausea can be delayed. Use the prescribed prevention and rescue plan; call if fluids will not stay down.

4 Blood counts / infection

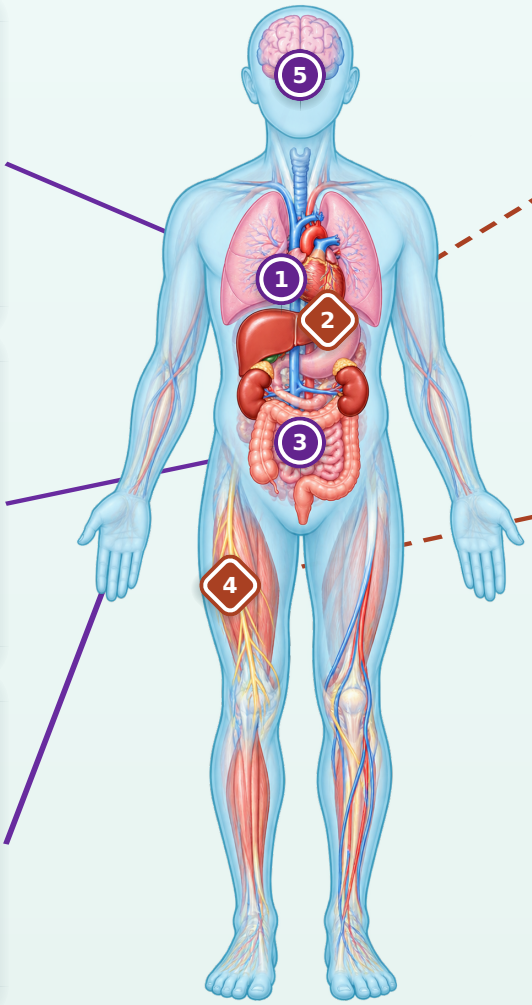
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Bloods are checked before each cycle. Fever, feeling suddenly unwell or unusual bleeding needs prompt contact.

5 Hair loss

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Hair thinning or loss can occur. Ask about scalp and practical support before treatment starts.



Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Contact the oncology team urgently for any new or worsening cough, breathlessness or fever: lung inflammation can start at any point and can become serious. Do not wait until the next infusion or scan. Also report fever or feeling acutely unwell, unusual bleeding, inability to keep fluids down, chest pain or new ankle swelling. Severe breathing difficulty or collapse needs emergency care. Severe chest pain, major breathing difficulty, collapse or stroke symptoms: 112 / 999.

YOUR PRACTICAL TREATMENT PLAN

Know the next step.

Taking treatment

One infusion on Day 1 of a 21-day cycle. The next infusion is normally three weeks later. A delayed appointment is rescheduled by the team; patients do not calculate catch-up doses.

The first infusion takes 90 minutes. Later infusions may take 30 minutes if the previous infusion was tolerated. Allow extra time for assessment, anti-sickness medicines and any observation. A cannula or an existing central line can be used according to the clinical plan; there is no home pump.

Checks and follow-up

Before the first dose: full blood count, kidney and liver tests, an echocardiogram or MUGA scan to measure heart pumping function, and a pregnancy test where relevant. The team checks the HER2 result, previous treatment, lung history and current breathing symptoms, and agrees how disease response will be assessed. Blood count, kidney and liver tests before each 21-day cycle. Review breathing, fatigue, nausea, intake and other symptoms at each visit. Repeat cardiac imaging every 12 weeks; assess new breathing symptoms immediately rather than waiting for the next planned scan.

The whole-treatment map shows the checks for this exact treatment setting.

Support between reviews

Prevent nausea before it starts

This regimen has high emetic risk. An antiemetic combination is prescribed before every infusion, with an agreed plan for delayed symptoms and breakthrough nausea. The dedicated NCCP trastuzumab-deruxtecan pathway should be checked by the prescriber and pharmacist.

Lung-safety plan

Carry the patient alert card and report new cough, fever or breathlessness promptly. Steroids used for suspected lung inflammation are prescribed and monitored by the clinical team; do not start them yourself from this information.

Blood-count support

Low blood counts can cause infection, anaemia or bleeding. The need for G-CSF or transfusion support depends on results, symptoms and previous complications; it is not automatic for every cycle.

Eating and energy

Small frequent meals and early treatment of nausea can help maintain intake. Tell the team about weight loss, constipation or diarrhoea so that support can be tailored.

Oncology daytime: Insert numbers here. **Out of hours:** Insert numbers here. Follow your own hospital’s urgent-care plan. Emergency: 112 / 999.

Your complete treatment reference

https://sactcheck.com/patient/00776/

Public information only. No personal information is encoded.